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TCGA-MK-A4N6-01A-PR

Redacted



Results

Collection Information

Collection Date

Collection Time

Resulting Agency

SURGICAL PATHOLOGY

PATHOLOGIC DIAGNOSIS

A. LYMPH NODE, RIGHT PAROTID, EXCISION:

- ONE LYMPH NODE POSITIVE FOR METASTATIC PAPILLARY THYROID CARCINOMA (1/1 LYMPH NODE)

B. THYROID, TOTAL THYROIDECTOMY:

- PAPILLARY THYROID CARCINOMA
- TWELVE (12) LYMPH NODES IDENTIFIED, EIGHT (8) LYMPH NODES POSITIVE FOR METASTATIC PAPILLARY THYROID CARCINOMA (8/12)

Specimen size: 8.7 cm in greatest dimension

Specimen weight: 160 grams

Tumor focality: Unifocal

Tumor size: 8.2 cm in greatest dimension

Histologic type: Papillary carcinoma, classical type

Histologic grade: G1 (well differentiated)

Margins: Margins uninvolved by carcinoma

Tumor capsular invasion: Present (to soft tissue, minimal extent)

Lymphovascular invasion: Not identified

Perineural invasion: Not identified

Extrathyroidal extension: Present

Pathologic Staging:

- pT3
- pN1B (see below C, D, E and F)
- Number examined: 77
- Number involved: 40
- Lymph node, extranodal extension: Present
- pMX

C. SOFT TISSUE, RIGHT PARATRACHEAL NECK, DISSECTION:

- TWO LYMPH NODES IDENTIFIED, TWO LYMPH NODES POSITIVE

ICD O-3

Carcinoma, Papillary Thyroid

Site:

826013

Thyroid, NOS

C73.9

4/11/29/12

FOR METASTATIC PAPILLARY THYROID CARCINOMA (2/2)
- SOFT TISSUE EXTENSION OF PAPILLARY THYROID CARCINOMA

D. SOFT TISSUE, LEFT PARATRACHEAL NECK, DISSECTION:
- TWO LYMPH NODES IDENTIFIED, TWO LYMPH NODES POSITIVE
FOR METASTATIC PAPILLARY THYROID CARCINOMA (2/2)
- EXTRACAPSULAR EXTENSION PRESENT IN ONE LYMPH NODE

E. SOFT TISSUE, RIGHT NECK, RADICAL DISSECTION:
- FIFTY-NINE (59) LYMPH NODES IDENTIFIED, TWENTY-SIX (26)
LYMPH NODES POSITIVE FOR METASTATIC PAPILLARY THYROID
CARCINOMA (26/59)

F. LYMPH NODE, PRETRACHEAL, EXCISION:
- ONE LYMPH NODE IDENTIFIED, POSITIVE FOR METASTATIC
PAPILLARY THYROID CARCINOMA (1/1)

Staff Pathologist

Intraoperative Consultation

Intraoperative Frozen Section Diagnosis

FSA, TPA: Lymph node, right parotid, excision:

- Positive for metastatic carcinoma (1/1 lymph node)

Pertinent Clinical Information
Papillary thyroid cancer.

Gross Description

Specimen Material: (A) Right parotid lymph node, (B) Total thyroidectomy,
right neck dissection, (C) Right paratracheal neck dissection, (D) Left
paratracheal neck dissection, (E) Right radial neck dissection, (F)
Pretracheal lymph node.

The case is received in six parts each labeled with the patient's name,
medical record number and given accession number
and it is accompanied by a requisition form labeled with the same
name and same accession number.

Part A: The specimen is received fresh for frozen section diagnosis labeled

"RIGHT PAROTID LYMPH NODE" and consists of a 2.5 x 2.0 x 1.0 cm, tan-brown, rubbery to soft, possible lymph node. The specimen is bisected to reveal a 1.1 x 0.7 cm tan-brown to red, variegated possible lymph node, with the remainder of the surface being yellow and irregular. Touch prep is performed. A representative portion of the possible lymph node is submitted for frozen section diagnosis as FSA1. The remainder of the specimen is submitted in its entirety in cassettes A2-A3.

Part B: The specimen is received fresh labeled with the patient's name and medical record number "TOTAL THYROIDECTOMY, RIGHT NECK DISSECTION" and consists of a 160 gram, 8.7 x 6.8 x 4.8 cm, unoriented total thyroidectomy specimen with attached soft tissue. The surface of the specimen is tan-pink to tan-purple, nodular and irregular. The specimen is unoriented and is noted to be markedly distorted. The entire surface of the specimen is inked in blue. Sectioning reveals a heterogenous mass measuring 8.2 x 5.5 x 4.6 cm. Approximately 65% of the mass displays a tan-brown to yellow, smooth to papilliferous cut surface with multifocal hemorrhage and necrosis, resulting in a soft consistency. The remaining portion of the mass consists of a tan-brown to yellow, smooth cut surface with a firm consistency. The mass displays a discontinuous tan-brown, fibrous capsule. The mass focally abuts the blue inked surgical margin. There is a minimal amount of remaining uninvolved thyroid parenchyma, which displays a beefy red cut surface. No parathyroid glands are identified grossly.

The specimen displays three areas of attached soft tissue; each located approximately 180 degrees from the other. These areas are arbitrarily designated soft tissue area 1, soft tissue area 2, and soft tissue area 3. Sectioning through area 1 reveals a yellow to tan-brown irregular to lobulated cut surface with multiple tan-brown, rubbery possible lymph nodes ranging from 0.2 to 1.2 cm in greatest dimension. Sectioning through area 2 reveals a 4.5 x 2.3 x 1.6 cm firm, tan-brown possible lymph node. Sectioning through area 3 reveals multiple tan-brown, rubbery, possible lymph nodes ranging from 0.4 to 1.2 cm in greatest dimension.

Representative sections are submitted as follows:

B1: tumor to soft tissue area 1

B1-3: tumor to soft tissue area 2

B4: tumor to soft tissue area 3

B5: multiple possible lymph nodes from area 1

B6: one possible lymph nodes from area 1, bisected

B7-10: one possible lymph node from soft tissue area 2, serially sectioned

B11: multiple possible lymph nodes from area 3

B12: one possible lymph node from area 3, bisected

B13: tumor to blue inked margin

B14: tumor to surrounding tissue

B15: tumor to blue inked margin

B16: friable portion of tissue
B17: representative solid portion of tumor
B18: representative solid portion of tumor
B19: tumor to capsule
B20: friable portion of tumor
B21: representative solid portion of tumor
B22: tumor to normal
B23: representative tumor
B24: uninvolved thyroid

Part C: Received in formalin labeled "RIGHT PARATRACHEAL NECK DISSECTION" and consists of a 3.1 x 3.1 x 1.4 cm aggregate of tan-brown to yellow, soft, irregular tissue which is sectioned and palpated to reveal multiple tan-brown, rubbery possible lymph nodes ranging from 0.4 to 1.9 cm in greatest dimension. Representative sections are submitted as follows:

C1: one possible lymph node, bisected
C2: one possible lymph node, bisected
C3: multiple intact possible lymph nodes

Part D: Received in formalin labeled with the patient's name and number "LEFT PARATRACHEAL NECK DISSECTION" and consists of a 3.2 x 2.6 x 0.9 cm aggregate of tan-brown to yellow, irregular to lobulated soft tissue which is sectioned and palpated to reveal two tan-brown, rubbery possible lymph nodes measuring 0.7 x 0.6 x 0.3 cm and 1.2 x 0.5 x 0.5 cm, respectively. Representative sections are submitted as follows:

D1: one possible lymph node, bisected
D2: one possible lymph node, bisected

Part E: Received in formalin labeled "RIGHT RADICAL NECK DISSECTION" and consists of an unoriented portion of fibro-fatty tissue measuring 16.9 x 13.2 x 3.1 cm, to include a partial sternocleidomastoid muscle (11.5 x 7.7 x 3.1 cm), partial jugular vein (4.9 cm in length and 0.4 cm in diameter), and attached soft tissue. No submandibular gland is identified. The specimen is arbitrarily designated into the following levels:

Level 1 (largest portion of muscle and adjacent adipose tissue)
Levels 2, 3, and 4 (presumed inferior portion of sternocleidomastoid muscle and jugular vein, with adjacent soft tissue)
No level 5 is identified

The sternocleidomastoid muscle is sectioned to reveal a beefy-red, striated, unremarkable cut surface. The adipose tissue from level 1 is sectioned and palpated to reveal multiple tan-brown, rubbery possible lymph nodes ranging from 0.2 to 0.8 cm in greatest dimension.

Level 2 is sectioned and palpated to reveal multiple tan-brown, rubbery possible lymph nodes ranging from 0.4 to 1.3 cm in greatest dimension.

Level 3 is sectioned and palpated to reveal multiple tan-brown, rubbery possible lymph nodes ranging from 0.7 to 1.8 cm in greatest dimension.

Level 4 is sectioned and palpated to reveal multiple tan-brown, rubbery possible lymph nodes ranging from 0.4 to 2.5 cm in greatest dimension.

Representative sections are submitted as follows:

E1-E2: representative sternocleidomastoid muscle

E3: representative jugular vein

E4: five possible level 1 lymph nodes

E5: seven possible level 1 lymph nodes

E6: six possible level 2 lymph nodes

E7: two possible level 2 lymph nodes, bisected, one inked in black and one inked in blue

E8: one possible level 2 lymph node, trisected

E9: six possible level 3 lymph nodes

E10: one possible level 3 lymph node, bisected

E11: four possible level 4 lymph nodes

E12: four possible level 4 lymph nodes

E13: one possible level 4 lymph node, bisected

E14: one possible level 4 lymph node, bisected

Part F: Received in formalin labeled "PARATRACHEAL LYMPH NODE" and consists of a 1.2 x 0.9 x 0.6 cm aggregate of yellow to tan-brown, soft, irregular tissue which is sectioned and palpated to reveal a 0.7 x 0.4 x 0.3 cm, tan-brown, rubbery possible lymph node. The specimen is submitted in its entirety in cassette F.

Pathology Resident

Microscopic Description

A: H&E-stained sections of the lymph node show metastatic papillary thyroid carcinoma with multiple psammoma bodies.

B: H&E-stained sections of the thyroid gland show a neoplasm consisting of branching papillae. The epithelium covering the papillae is comprised of well differentiated, uniform, orderly cuboidal cells. The neoplastic cells display finely dispersed chromatin with intranuclear pseudoinclusion and optically clear nucleus with eccentric nucleolus (Orphan Annie appearance). In addition, occasional nuclear grooves are seen. Scattered psammoma bodies are identified. The tumor exhibits extracapsular extension, invading into the surrounding soft tissue, with nests of tumor focally surrounded by dense fibrosis. The tumor approaches the inked margin; however, the margin is free of carcinoma. Focally, cystic change is present within the neoplasm. In addition, foreign bodies consistent with suture material are identified, and are surrounded by reactive fibrosis. The residual uninvolved thyroid

parenchyma is unremarkable. No lymphovascular or perineural invasion is identified (despite multiple tumor metastases in the lymph nodes). Within the surrounding soft tissue, twelve lymph nodes are identified, and eight of the lymph nodes are positive for metastatic papillary thyroid carcinoma (also see below C, D, E and F).

C: H&E-stained sections show two lymph nodes with surrounding soft tissue. Metastatic papillary thyroid carcinoma is present in both lymph nodes. There is metastatic papillary thyroid carcinoma present within the soft tissue.

D: H&E-stained sections show two lymph nodes, both of which are positive for metastatic papillary thyroid carcinoma. One of the lymph nodes shows extracapsular extension of the carcinoma.

E: H&E-stained sections show fifty-nine lymph nodes. Twenty-six lymph nodes are positive for metastatic papillary thyroid carcinoma. Focally, extracapsular extension is present. There is occasional cystic change within the lymph nodes. The sternocleidomastoid muscle, jugular vein, and surrounding soft tissue are otherwise unremarkable.

F: H&E-stained sections show one lymph node, which is positive for metastatic papillary thyroid carcinoma.

This case was reviewed by the staff pathologist listed below.

Pathology Resident

Electronically Signed Out

Staff Pathologist

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
HIFAA Discrepancy		☒
Prior Malignancy History		☒
Dual/Synchronous Primary Noted		☒
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials: BTL	Date Reviewed: 9/19/12	2/2/12